

Interactions, Allergies, Contraindications, and Duplicate Therapy Policy

Structured reconciliation and mandatory review controls

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Owner: Lead Cardiology Pharmacist
Approved by: Fictional Clinical Governance Committee
Classification: Training / synthetic corpus

Document control and retrieval metadata

Field	Value
Canonical title	Interactions, Allergies, Contraindications, and Duplicate Therapy Policy
Document ID	NB-MED-203
Keywords	interaction, allergy, intolerance, contraindication, duplication, reconciliation, openFDA
Authority	Lead Cardiology Pharmacist
Applicability	Training simulations only
Relationship	Standalone extension; existing corpus documents remain unchanged

Retrieval rule

Retrieve for every medication review and whenever a medicine, allergy, reaction, interacting product, condition, or duplicate class is present or uncertain.

1. Structured allergy assessment

Field	Requirement
Substance	Specific medicine/substance and formulation where relevant
Classification	Allergy, intolerance, adverse effect, contraindication, or unknown
Reaction	Observed symptoms, not only 'allergic'
Severity	Recorded category and clinical consequences
Timing	Date/age, latency, and most recent exposure if known
Certainty/source	Patient, carer, clinician, prior record, test, or unknown
Status	Active, disproven, resolved, or requires clarification
Cross-reactivity note	Only from approved clinical/pharmacy review

No allergy entry is not equivalent to 'no known allergies'. Unknown or conflicting allergy status prevents final authorization when relevant.

2. Interaction categories

Category	Examples of evidence to check	Required action
Drug-drug	Active, held, recently stopped, OTC and herbal products	Severity/mechanism/evidence from approved source; pharmacist review when material
Drug-condition	Renal/hepatic status, conduction/rhythm, bleeding, pregnancy relevance	Prescriber/pharmacist review; no model-only inference
Drug-laboratory	Potassium, renal function, sodium, haemoglobin, liver tests, ECG intervals	Use timestamp/unit/status and authorized thresholds
Drug-procedure	Surgery, dental work, fasting, contrast, device procedure	Peri-procedure plan required
Drug-food/substance	Food, alcohol, tobacco, grapefruit or supplements where approved source applies	Patient-specific counselling copied from current source/order
Therapeutic duplication	Same ingredient, overlapping class, brand/generic duplication	Reconcile before final recommendation

3. Interaction evidence workflow

- Normalize medicine identity without merging formulations automatically.
- Query approved interaction/label sources and record source, version, retrieval time, severity language, and uncertainty.
- Distinguish theoretical, reported, contraindicated, and monitoring-required evidence.

- Assess patient relevance using validated structured facts; the model may explain but not decide authorization.
- Route material findings to pharmacist/prescriber; record decision and monitoring.
- If a required interaction source is unavailable, return an incomplete review rather than 'no interactions found'.

4. openFDA limitation

openFDA drug labels can supply labeling text but are not treated as a complete interaction checker, prescribing authority, or patient-specific recommendation engine. A missing label section or search result is not evidence of no risk.

5. Duplicate and discrepancy rules

Finding	Disposition
Same active ingredient twice	Hold finalization; determine intentional versus duplicate
Brand and generic names appear separately	Normalize identity, retain provenance, verify formulation/strength
Active and stopped records conflict	Pharmacist reconciliation; do not choose newest solely by timestamp
Patient report conflicts with record	Mark unresolved; verify pharmacy/order/prescriber source
Formulation differs	Treat as potentially clinically material; no equivalence assumption
Dose expressed in incompatible units	No comparison or conversion without approved deterministic rule

6. Required output

- Allergy status and material reactions.
- Interaction findings with source and patient relevance.
- Contraindication/precaution findings.
- Duplicate or discrepancy findings.
- Authorization impact and review status.
- Required monitoring, owner, and timeframe.
- Safest next action when unresolved.

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